Vascular disorders
Ankle oedema, increase of an existing intermittent claudication, postural hypotension, cold or cyanotic extremities, Raynaud's phenomenon, paraesthesia of the extremities.
Respiratory, thoracic and mediastinal disorders
Bronchospasm (in patients with asthma or a history of asthma), nasal congestion, interstitial lung disease.
Gastrointestinal disorders
Epigastric pain, nausea, vomiting, diarrhoea.
Hepato-biliary disorders
Raised liver function tests, jaundice (both hepatocellular and cholestatic), hepatitis and hepatic necrosis.
Skin and subcutaneous tissue disorders
Sweating, tingling sensation in the scalp, usually transient, may occur in a few patients early in treatment, reversible lichenoid rash, systemic lupus erythematosus, exacerbation of psoriasis.
Musculoskeletal, connective tissue and bone disorders:
Cramps, toxic myopathy.
Renal and urinary disorders
Acute retention of urine, difficulty in micturition.
Reproductive system and breast disorders
Ejaculatory failure.
General disorders and administration site conditions
Hypersensitivity (rash, pruritus, angioedema and dyspnoea), drug fever, masking of the symptoms of thyrotoxicosis or hypoglycaemia, reversible alopecia.
Reproductive System and Breast disorders:
Frequency "not known": nipple pain, Raynaud´s phenomenon of the nipple

OVERDOSAGE
Symptoms of overdosage are bradycardia, hypotension, bronchospasm and acute cardiac insufficiency.
After an overdose or in case of hypersensitivity, the patient should be kept under close supervision and be treated in an intensive-care ward. Artificial respiration may be required. Bradycardia or extensive vagal reactions should be treated by administering atropine or methylatropine. Hypotension and shock should be treated with plasma/plasma substitutes and, if necessary, catecholamines. The beta-blocking effect can be counteracted by slow intravenous administration of isoprenaline hydrochloride, starting with a dose of approximately 5mcg/min, or dobutamine, starting with a dose of approximately 2.5mcg/min, until the required effect has been obtained. If this does not produce the desired effect, intravenous administration of 8-10 mg glucagon may be considered. If required the injection should be repeated within one hour, to be followed, if necessary, by an iv infusion of glucagon at 1-3 mg/hour. Administration of calcium ions, or the use of a cardiac pacemaker, may also be considered.
Oliguric renal failure has been reported after massive overdosage of labetalol orally. In one case, the use of dopamine to increase the blood pressure may have aggravated the renal failure.
Labetalol does have membrane stabilising activity which may have clinical significance in overdosage.
Haemodialysis removes less than 1% labetalol hydrochloride from the circulation.

STORAGE
Do not store above 30°C.
After dilution
Storage condition: between 2°C-8°C or below 30°C
Shelf life: 24hrs.

INSTRUCTION FOR USE
Compatible diluents: Ringer, Ringer's lactate, 5 % Dextrose , 0.9 % Sodium chloride
Blocamine were diluted into proposed media, in order to achieve a final concentration of 1 mg/ml of labetalol.
After dilution product must be free from foreign particles.

SHELF LIFE
24 months.

Therapeutic code /ATC code
C07AG01

HOW SUPPLIED
Carton box containing 10 ampoules of 4 ml.
KEEP THIS AND ALL MEDICINES OUT OF REACH OF CHILDREN. This drug has been prescribed for YOUR CURRENT MEDICAL PROBLEM, DO NOT RECOMMEND IT TO OTHERS. THIS MEDICINE MUST BE USED UNDER PRESCRIPTION AND UNDER MEDICAL SUPERVISION. CAN NOT BE REPEATED WITHOUT A NEW PRESCRIPTION.

MAL No: MAL19096027ACRZ

Technical director: Gustavo Omar Sein.
Pharmacist and bachelor in pharmaceutical sciences.

Product Owner:
Lafedar S.A. Valentín Torrá 4880, General Belgrano Industrial Park, Paraná, Entre Ríos, Argentina.

Product Manufacturer:
INSTITUTO BIOLOGICO CONTEMPORANEO S.A.
Chivilcoy 304 and Bogotá 3921/25
(Zip Code C1407ESM), Autonomous City of Buenos Aires, Argentina.

Product Registration Holder:
Healol Pharmaceuticals Sdn Bhd.
No 74-3, Jalan Wangsa Delima 6, KLSC Wangsa Maju, 53300 Kuala Lumpur, Malaysia.

Revision Date October 2022

LAFEDAR

00-0000/A

BLOCAMINE®
Labetalol Hydrochloride 5 mg/ml
Solution for Injection and for Infusion

LAFEDAR
Sale under prescription
Made in Argentina

FORMULA
Each ml of solution for injection contains: Labetalol hydrochloride 5 mg, Excipients: Anhydrous dextrose , Disodium edetate, Methylparaben, propylparaben, Sodium hydroxide and/or citric acid, water for injection e.q. 1.00 ml

PRODUCT DESCRIPTION
Limpid solution without foreign matter. After dilution product must be free from foreign particles.

INDICATIONS
- Severe hypertension, including severe hypertension of pregnancy, when rapid control of blood pressure is essential.
-Anaesthesia when a hypotensive technique is indicated.
-Hypertensive episodes following acute myocardial infarction.

ADMINISTRATION ROUTE
Blocamine injection is destined to be administered intravenously as bolus or infusion in hospitalized patients.

PROPERTIES
Pharmacological action:
Blocamine has α and β blocking properties. It reduce blood pressure by blocking the α-adrenergic receptors of the peripheral arteries (vasodilation) reducing peripheral resistance. At the same time, Blocamine blocks the β- adrenergic receptors, protecting the heart from sympathetic response as that may cause peripheral vasodilation by blocking the receptors α. This gives a lower blood pressure without cardiac stimulation. Blocamine has low β-sympathomimetic intrinsic activity.
Pharmacokinetic and metabolism:
The plasma half-life of labetalol is about 4 hours. About 50% of labetalol in the blood is protein bound. Labetalol is metabolised mainly through conjugation to inactive glucuronide metabolites. These are excreted both in urine and via the bile into the faeces.
Only negligible amounts of the drug cross the blood brain barrier in animal studies.

DOSAGE / ADMINISTRATION
Labetalol hydrochloride injection is intended for i.v. use in hospitalised patients. Patients should always receive the drug whilst in the supine or left lateral position. Raising the patient into the upright position within 3 h of i.v. labetalol hydrochloride administration should be avoided since excessive postural hypotension may occur. It is desirable to monitor the blood pressure and heart rate after injection and during infusion. In most patients, there is a small decrease in the heart rate; severe bradycardia is unusual but may be controlled by injecting atropine 1 to 2 mg intravenously. Respiratory function should be observed particularly in patients with any known impairment.
Once the blood pressure has been adequately reduced by bolus injection or infusion, maintenance therapy with labetalol hydrochloride tablets should be substituted with a starting dose of 100 mg twice daily.

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(Fecha)	(Fecha)	(Fecha)	(Fecha)	(Fecha)

Labetalol hydrochloride injection has been administered to patients with uncontrolled hypertension already receiving other hypotensive agents, including beta-blocking drugs, without adverse effects.
Adults
For Severe Hypertension

Bolus Injection
If it is essential to reduce the blood pressure quickly a dose of 50 mg should be given by i.v. injection (over a period of at least 1 min) and, if necessary, repeated at 5 min intervals until a satisfactory response occurs. The total dose should not exceed 200 mg. The maximum effect usually occurs within 5 min and the duration of action is usually about 6 h but may be as long as 18 h.

Intravenous Infusion
A 1mg/ml solution of Labetalol hydrochloride injection should be used, i.e. the contents of ten 4 ml ampoules (200 mg) diluted to 200 ml with proposed media.
Compatible diluents: Ringer, Ringer's lactate, 5 % Dextrose, 0.9 % Sodium chloride.
After dilution product must be free from foreign particles.

-Severe Hypertension of Pregnancy
Infusion should be started at 20 mg/h, then doubled every 30 min until a satisfactory response is obtained or a dosage of 160 mg/h is reached. Occasionally higher doses may be necessary.

-Hypertension Due To Other Causes
Infusion should be started at a rate of 2 mg/min until a satisfactory response is obtained, then stopped. The effective dose is usually 50 to 200 mg but larger doses may be needed, especially in patients with phaeochromocytoma. The rate of infusion may be adjusted according to the response at the discretion of the physician.

For Hypotensive Anaesthesia
Induction should be with standard agents (e.g. sodium thiopentone) and anaesthesia maintained with nitrous oxide and oxygen with or without halothane. The recommended starting dose of labetalol hydrochloride is 10 to 20 mg intravenously depending on the age and condition of the patient. Patients for whom halothane is contraindicated usually require a higher initial dose of labetalol hydrochloride (25 to 30 mg). If satisfactory hypotension is not achieved after 5 min, increments of 5 to 10 mg should be given until the desired level of blood pressure is attained.

Halothane and labetalol hydrochloride act synergistically therefore the halothane concentration should not exceed 1 to 1.5% as profound falls in blood pressure may be precipitated. Following labetalol hydrochloride injection the blood pressure can be quickly and easily adjusted by altering the halothane concentration and/or adjusting table tilt. The mean duration of hypotension following 20 to 25 mg of i.v. labetalol hydrochloride is 50 min. Hypotension induced by labetalol hydrochloride injection is readily reversed by atropine 0.6 mg and discontinuation of halothane.

Tubocurarine and pancuronium may be used when assisted or controlled ventilation is required. Intermittent positive pressure ventilation may further increase the hypotension resulting from labetalol hydrochloride injection and/or halothane.

For Hypertensive Episodes Following Acute Myocardial Infarction
Infusion should be started at 15 mg/h and gradually increased to a maximum of 120 mg/h depending on the control of blood pressure.

CONTRAINDICATIONS
BLOCAMINE is contraindicated in patients with hypersensitivity to labetalol or any of the excipients.
BLOCAMINE is contraindicated in atrioventricular block of second and third degree, cardiogenic shock and other conditions associated with severe and prolonged hypotension or severe bradycardia.
Do not use β-blockers, even seemingly cardioselective, if there is a history of asthma or airway obstruction.
BLOCAMINE is contraindicated to control hypertensive episodes following acute myocardial infarction, when there is peripheral vasoconstriction that suggest low cardiac output.

WARNINGS
Special care is indicated in patients with heart failure or left ventricular failure. In heart failure, the disease must be treated properly before starting treatment with BLOCAMINE. Abrupt discontinuation of treatment with β-blockers must be avoided, especially in cases of myocardial ischemia. BLOCAMINE must be suspended gradually by reducing the dose increases. No need to interrupt management with BLOCAMINE before practice narcosis, but the patient will receive before an intravenous injection of atropine. BLOCAMINE potentiates the action of halothane on blood pressure.
The following effects have been noted in isolated cases after treatment with elevated short-term or long-term parameters of liver function, jaundice (due to a disturbance of liver function or cholestasis), hepatitis, liver necrosis. In most cases, these problems have been resolved spontaneously after discontinuation of labetalol.
Appropriate essential laboratory tests at the first sign of liver failure. If the results indicate liver dysfunction or appearance of jaundice, BLOCAMINE should be discontinued immediately and not be restarted.
If a patient treated with β-blockers has a history of severe anaphylactic reactions due to of all types allergens, special care in re-exposure to β-blockers must be taken. These patients may not respond to the usual doses of epinephrine used to treat allergic reactions.
In the case of peripheral vascular disease, BLOCAMINE must be used with caution, since it is possible the worsening of symptoms.
If symptomatic bradycardia occurs, reduce the dose of BLOCAMINE.
Because β-blockers have a negative effect on atrioventricular conduction BLOCAMINE should be used with caution in patients with first degree atrioventricular block.
Like other β-blockers, BLOCAMINE can mask the symptoms of hyperthyroidism and hypoglycemia in diabetic patients.
If patients already receiving labetalol require more adrenaline treatment, the dose of adrenaline must be reduced for simultaneous administration with labetalol because adrenaline can cause bradycardia and hypertension.
In patients with pheochromocytoma, labetalol can be administered only after obtaining a suitable block of alpha-adrenergic receptors.
BLOCAMINE can hide compensatory physiological responses of the body to a sudden haemorrhage (tachycardia and vasoconstriction). Therefore, it is possible to carefully monitor blood loss during surgery.
BLOCAMINE must be used with caution in patients with hepatic impairment, since the metabolism of labetalol in this group of patients is slow compared to that observed in healthy persons.
During cataract surgery in some patients who were treated with tamsulosin, the appearance of floppy iris syndrome (IFIS, a variant of small pupil syndrome) was observed.

INTERACTIONS
BLOCAMINE produces fluorescence in alkaline solution at excitation wavelength of 334 nm and 412 nm of fluorescence wavelength, therefore may interfere with the assessments of certain fluorescent substances including catecholamines.
The presence of labetalol metabolites in urine can produce falsely elevated catecholamine levels, metanephrine, normetanephrine and vanillylmandelic acid (VMA) to be determined by fluorimetric or photometric methods. In Controlled patients, suspected of having that pheochromocytoma and treated with Blocamine hydrochloride, should be employed for determining levels of catecholamines, a specific method as high performance liquid chromatography solid phase pressure extraction.
The β-blockers should not be administered concurrently with the following drugs: Class I antiarrhythmic drugs (eg, quinidine and analogues, lidocaine and similar) and Class IV antiarrhythmic agents (calcium antagonist verapamil type).
Concomitant use of tricyclic antidepressants can lead to an increased frequency of tremors.
Cimetidine can increase the bioavailability of orally administered labetalol.
Inhibitors of prostaglandin synthesis, such as ibuprofen and indomethacin can reduce the antihypertensive effect of Labetalol. Therefore, the antihypertensive effect of Blocamine may be decreased during the simultaneous administration of inhibitors of prostaglandin synthesis (NSAID, for example), which may require an adjustment of dose.
It is established that the labetalol reduces binding of the radioisotope MIBG type (metaiodobenzylguanidine), therefore the results of the analyses performed with MIBG must be interpreted with caution
The negative chronotropic effect of digoxin can be increased by labetalol administration.
Concomitant administration of labetalol and adrenaline can cause bradycardia and hypertension.
Labetalol hydrochloride injection may enhance the hypotensive effects of halothane.

Effects on ability to drive and use machines:
There are no studies on the effect of this medicine on the ability to drive. When driving vehicles or operating machines it should be taken into account that occasionally dizziness or fatigue may occur.

Fertility, pregnancy and lactation
In women, labetalol crosses the placenta, so it is necessary to consider the possible consequences of a blockage of the α and β-adrenergic receptors in the fetus and newborn. In rare cases they have been reported perinatal and neonatal bradycardia, hypotension, respiratory depression, hypoglycaemia and hypothermia. These symptoms may appear one or two days after birth. BLOCAMINE passes in small amounts to breast milk (about 0.004% of the maternal dose).
Nipple pain and Raynaud's phenomenon of the nipple have been reported.

ADVERSE REACTIONS
Labetalol Injection is usually well tolerated. Excessive postural hypotension may occur if patients are allowed to assume an upright position within three hours of receiving Labetalol Injection.
Most side-effects are transient and occur during the first few weeks of treatment with labetalol.
They include:
Blood and the lymphatic system disorders
Rare reports of positive antinuclear antibodies unassociated with disease, hyperkalaemia, particularly in patients who may have impaired renal excretion of potassium, thrombocytopenia.
Psychiatric disorders
Depressed mood and lethargy, hallucinations, psychoses, confusion, sleep disturbances, nightmares.
Nervous system disorders
Headache, tiredness, dizziness, tremor has been reported in the treatment of hypertension of pregnancy.
Eye disorders
Impaired vision, dry eyes.
Cardiac disorders
Bradycardia, heart block, heart failure, hypotension.

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